

RIVERSIDE MULTISPECIALTY HOSPITAL

Department of Endocrinology

Initial Consultation Note

Patient Name:	John Doe	Medical Record #:	MRN-2026-001247
Age / Sex:	54 years / Male	Date of Note:	2026-01-15
Consultant:	Dr. Sarah Smith, MD (Endocrinology)		

CHIEF COMPLAINT

Patient presents with increased thirst, frequent urination, fatigue, and gradual weight gain over the past several months.

HISTORY OF PRESENT ILLNESS

Mr. John Doe reports experiencing excessive thirst (polydipsia), increased frequency of urination (polyuria), and persistent fatigue for approximately four months. He also reports occasional blurred vision during evening hours.

He denies chest pain, shortness of breath, fever, recent infections, or significant weight loss. Family history is significant for Type 2 Diabetes Mellitus in his father.

PAST MEDICAL HISTORY

- No known chronic illnesses
- No previous surgeries
- No known drug allergies

PHYSICAL EXAMINATION & VITALS

Parameter	Value	Reference / Assessment
Blood Pressure	138/86 mmHg	Prehypertension / Borderline Elevated
Heart Rate	82 bpm	Normal (60-100 bpm)
Respiratory Rate	16 / min	Normal (12-20 / min)
Temperature	98.4 °F	Normal
Height / Weight	173 cm / 92 kg	-
BMI	30.7 kg/m²	Obesity (Class I)

General appearance: Alert and oriented. No acute distress.

LABORATORY FINDINGS

Test Description	Result	Reference Range
HbA1c	8.6%	< 5.7% (Normal)
Fasting Plasma Glucose	186 mg/dL	70 - 100 mg/dL
Random Blood Glucose	241 mg/dL	< 140 mg/dL
Serum Creatinine	0.9 mg/dL	0.7 - 1.3 mg/dL
eGFR	>90 mL/min/1.73m ²	>60 mL/min/1.73m ²

ASSESSMENT

- **Newly diagnosed Type 2 Diabetes Mellitus** - supported by classic symptoms (polyuria, polydipsia), an HbA1c of 8.6%, and markedly elevated fasting/random plasma glucose metrics.
- **Obesity (BMI 30.7)** - contributing factor to insulin resistance.
- Elevated blood glucose requiring prompt pharmacologic therapy and lifestyle modification.

TREATMENT PLAN

- **Pharmacotherapy:** Initiate Metformin 500 mg orally twice daily.
- **Dietary:** Refer for formal diabetic diet counseling.
- **Physical Activity:** Recommend brisk walking at least 30 minutes daily.
- **Weight Management:** Encourage gradual and sustainable weight reduction.
- **Monitoring:** Perform daily home blood glucose monitoring (fasting and post-prandial).
- **Follow-up Labs:** Repeat HbA1c in 3 months.

FOLLOW-UP

Follow-up appointment scheduled after 12 weeks with repeat laboratory investigations as outlined above.

Electronically Signed By:
Dr. Sarah Smith, MD
Consultant Endocrinologist
Riverside Multispecialty Hospital